



February 23, 2026

Thomas Keane, MD, MBA
 Assistant Secretary for Technology Policy
 Office of the Deputy Secretary and Assistant Secretary for Technology Policy
 Department of Health and Human Services
 Mary E. Switzer Building; Mail Stop: 7033A
 330 C Street S.W.
 Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Dr. Keane,

Vantage Precision Health (VPH) is grateful for the opportunity to submit comments in response to the Department of Health and Human Services' (HHS) Request for Information (RFI) regarding the acceleration of artificial intelligence (AI) adoption as part of clinical care, published in the Federal Register at 90 FR 60108 on December 23, 2025. Vantage Precision Health is a consulting and advisory firm specializing in AI adoption strategy and program evaluation for behavioral health and aging services providers. We work directly with community mental health centers, substance use treatment organizations, psychiatric rehabilitation programs, and providers of long-term care for older adults — the very organizations that would benefit profoundly from thoughtful AI integration, and that *face the greatest risks if that integration is poorly designed, inadequately evaluated, or prematurely accelerated.*

Our perspective is shaped by daily engagement with providers who operate at the intersection of chronic underfunding, and the collision of growing demand for services combined with workforce shortages. These providers serve populations — people living with serious mental illness, substance use disorders, co-occurring conditions, and age-related cognitive and functional decline — whose needs are complex, whose vulnerabilities are significant, and whose voices are too often absent from national technology policy conversations. We offer these comments in the hope that HHS will center the experiences of these populations and their providers as it develops its AI policy framework.

Our Overarching Position

Vantage Precision Health strongly supports the responsible adoption of AI in clinical care and believes that well-designed, rigorously evaluated AI tools can meaningfully

expand access, reduce administrative burden, improve care coordination, and advance health equity in behavioral health and aging services. We are, however, deeply concerned that the current policy environment — characterized by a lack of uniform federal standards, inadequate reimbursement pathways, and a dearth of independent outcome research — creates conditions in which the promise of AI may be undermined by premature deployment, vendor-driven evaluation, and the exclusion of safety-net behavioral health providers from the innovation ecosystem.

We therefore urge HHS to pursue a balanced approach that simultaneously accelerates access to high-value AI tools and establishes the regulatory, reimbursement, and research infrastructure necessary to ensure that those tools are safe, effective, and equitable. Acceleration without this infrastructure will not serve patients, providers, or the public interest.

Behavioral Health and Aging Services: A Distinct Policy Context

Behavioral health and aging services providers occupy a unique and underserved position in the health information technology landscape. Because these providers were largely excluded from the Health Information Technology for Economic and Clinical Health (HITECH) Act of 2009, many lack the electronic health record (EHR) infrastructure, interoperability capacity, and data governance frameworks that form the foundation for AI integration. The costs of AI solutions remain prohibitive for many community-based behavioral health organizations, in part because AI vendors must build compatibility across dozens of disparate EHR systems — a burden that is passed directly to providers. Sustainable public financing for AI adoption in this sector is not yet in place, and reimbursement pathways for AI-assisted services remain unclear.

At the same time, the populations served by behavioral health and aging services providers present distinctive clinical and ethical considerations for AI deployment. The therapeutic relationship is a primary driver of outcomes in mental health treatment and the talk-therapy components of addiction treatment, and AI tools that supplant rather than support that relationship risk undermining care quality. Additionally, older adults, particularly those with cognitive impairment, may face heightened risks from AI systems that are not designed with their needs and capacities in mind (similar to what occurred during the COVID-19 pandemic with many long-term care residents unable to communicate effectively with family and friends via smartphones or tablets). And the intersection of AI with sensitive behavioral health data — including information protected under 42 CFR Part 2 — raises privacy and confidentiality concerns that require specific regulatory attention.

Key Recommendations

Uniform standards for AI safety, privacy, and efficacy. HHS should establish uniform federal standards for AI safety, privacy, and efficacy that are specifically calibrated to the behavioral health and aging services context. These standards should address the

unique confidentiality requirements of substance use disorder treatment records, the clinical and ethical dimensions of AI use with populations experiencing serious mental illness or cognitive decline, and the interoperability gaps that continue to disadvantage safety-net providers.

Reimbursement pathways for AI tools in BH and AS. HHS should develop and fund reimbursement pathways that enable behavioral health and aging services providers to adopt high-value AI tools — particularly those that reduce administrative burden, support care coordination, and enhance clinical decision-making — without bearing unsustainable implementation costs. This should include reimbursement for provider training and patient education, recognition of the workforce investment required for responsible AI adoption, and incentives for AI tools that demonstrate measurable improvements in health-related quality of life rather than engagement metrics alone.

Oversight and funding of outcomes research with healthcare AI tools. HHS should invest in independent, rigorous outcomes research on AI in behavioral health and aging services, conducted by researchers without financial ties to AI vendors. The evidence base for AI in these settings remains nascent—especially in behavioral health—and the metrics used by commercial developers — e.g., downloads, user engagement, and customer satisfaction — are not adequate substitutes for clinical outcome data. HHS should prioritize research that examines the impact of AI tools such as therapy chatbots on recovery, functional status, and quality of life, and that attends to differential effects across populations defined by race, ethnicity, age, disability status, and socioeconomic circumstances.

Uniform data and software standards. HHS should oversee and fund the development of uniform data and software standards for healthcare AI. Nascent tools, e.g. ambient-listening AI scribes, should be tested using the same representative, minimally biased data. The AI tools should also be required to be *interoperable*, as the lack of this requirement for EHRs and associated databases has produced decades of difficulty with adoption and use. This has especially affected behavioral health providers, who were not included in the HITECH Act’s funding pool and have lagged far behind the rest of medicine in the development of EHRs and other specialized technologies for mental health and substance use disorder treatment.

Engagement of providers and patients in shaping AI data and tool governance. HHS should ensure that behavioral health and aging services providers, and the patients and communities they serve, have a meaningful voice in AI governance. This includes representation on any technical expert panels convened by ASTP/ONC, inclusion in pilot programs and cooperative agreements, and active outreach to organizations serving populations that are historically underrepresented in health technology policy.

Non-medical device status. Currently, AI scribes and many other AI tools impacting clinical care are not recognized as medical devices by the FDA. This classification is critical for adequate oversight and needs to be changed. In the case of AI scribes,

behavioral health has unique requirements for them because therapy is almost exclusively *language-based*, and some of the most critical patient communication lies in ambiguity, not explicitness. The architecture and training of AI scribes to write concise, fluent notes impedes their correct interpretation of many ambiguous or nuanced, yet critical, behavioral health utterances (e.g., “I don’t know how much longer I can go on” reflecting possible suicidal ideation and “I just couldn’t stand it anymore” signaling a potential relapse).

Conclusion

Vantage Precision Health is committed to advancing the responsible and equitable adoption of AI in behavioral health and aging services. We believe that HHS has a historic opportunity to shape an AI policy framework that expands access, improves outcomes, and builds public trust — and we are eager to contribute our expertise to that effort. We would welcome the opportunity to participate in any technical panels, listening sessions, or collaborative research initiatives that ASTP/ONC convenes in connection with this RFI, and we are available to provide additional information or clarification at any time.

Thank you for your careful consideration of our comments.

Respectfully,

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Vantage Precision Health is a consulting and advisory firm specializing in AI adoption strategy and program evaluation for behavioral health and aging services providers. The firm works with community mental health centers, substance use treatment organizations, psychiatric rehabilitation programs, and home- and community-based services providers for older adults to design, implement, and evaluate technology-enabled programs that improve care quality, expand access, and advance health equity.